PEER REVIEW HISTORY

BMJ Open publishes all reviews undertaken for accepted manuscripts. Reviewers are asked to complete a checklist review form (**http://bmjopen.bmj.com/site/about/resources/checklist.pdf**) and are provided with free text boxes to elaborate on their assessment. These free text comments are reproduced below.

ARTICLE DETAILS

TITLE (PROVISIONAL)	Experiences and views of receiving and delivering information about recovery in acquired neurological conditions: A systematic review of qualitative literature
AUTHORS	Burton, Louisa-Jane; Forster, Anne; Johnson, Judith; Crocker, Thomas; Tyson, Sarah; Wray, Faye; Clarke, David

VERSION 1 – REVIEW

REVIEWER	Andrew Soundy University of Birmingham
REVIEW RETURNED	19-Oct-2020

GENERAL COMMENTS	Generally, this is an interesting area of study. But I think there are major questions I have, I have listed these below. First, I think there is a lack of consideration to differences that may exist between groups, I think there is a danger that the reader applies all findings across groups included. I think there will be a lot more articles that reported meeting your current eligibility criteria that have not been identified. I think as a reader I need to be convinced of the value further and would be interested in your responses to the comments. Introduction My main concern is that there is a lot of reviews on stroke/SCI/brain injury etc with will contain overlapping information e.g., just in stroke literature there is a lot of past reviews. Within the literature there are so many ways the groups could be considered in past evidence it's hard to identify how novel your work is e.g., From a 30 second search on google scholar these reviews looks at HCPS and communication in neuro https://www.tandfonline.com/doi/abs/10.1080/09638288.2020.1822935 https://www.sciencedirect.com/science/article/pii/S0738399120301816 https://www.tandfonline.com/doi/abs/10.1080/09638288.2018.1473508 https://link.springer.com/article/10.1186/s13012-017-0599-3 so that is just one group with out a focus on conditions. Other reviews cover aspects too https://www.tandfonline.com/doi/abs/10.3109/11038128.2010.509887 has a theme on information needs https://onlinelibrary.wiley.com/doi/full/10.1111/j.1369-7625.2011.00683.x has themes related to aspect of care and communication

	https://journals.plos.org/plosone/article?id=10.1371/journal.pone.0192533 you have other reviews which consider some of the issues you are talking about in other ways e.g., https://www.magonlinelibrary.com/doi/full/10.12968/ijtr.2014.21.5.210 in terms of the focus there is areas you consider within the results which again represent more literature e.g., there is hope (how that is defined varies), expectation (different form hope), recovery (this has its own focus), interactions and communication, goal setting – all of these areas could overlap with your findings. Methods Should a framework like ENTREQ be employed? Paradigmatic view of the world needs to be added and make sure this links to your quality text? From experience of reviewing qualitative neurological literature most qualitative articles that consider the experience of the condition will include information that meets your eligibility criteria – this means there is likely a lot more than 28 articles that could be eligible - how do you convince me that this is not the case? An audit trail to show the synthesis stages to it can be followed? The quality assessment – does it impact the synthesis results – if so how can you let the reader know the impact on the themes so what if a theme has weak studies? You rate studies by the ability to fulfil checklist but so what? – does it mean anything for themes? Results How does your results account for the various conditions - are you implying that all conditions have the same problems? Could there be a consistent way to identify if the examples you give are from certain conditions, times during the patient journey, delivered by a certain HCP group? Or are you saying all information applies to all groups all the time?

REVIEWER	Dr Stephanie Kilinc Teesside University UK
REVIEW RETURNED	03-Nov-2020

GENERAL COMMENTS	This is a very interesting review and I particularly like the way that the team have combined patient, caregiver and HCP data in the review. The relevant quality procedures have taken place for a qualitative systematic review and the analysis is well detailed and discussed. I only have a few queries, mainly for clarification purposes: 1. My main query is around what the team are classifying as neurological conditions. Specifically, they report that the papers in the review covered stroke, TBI, SCI, brain tumours and neurological conditions and I would like to know which neurological conditions were included in the review findings. Also, the title and aim of the paper refer to acquired neurological

	conditions, so it seems a little odd to me that they would then separate some specific neurological conditions in the findings but group others together as neurological conditions. Similarly, in the first theme, the authors discuss how information was often provided during rehabilitation. However, not all people with neurological conditions require or receive rehabilitation (e.g. some forms of epilepsy), so again, some further context regarding the general neurological conditions included in the review would be useful to determine if some patients may be missing out from receiving further information as a result of not necessarily being referred to rehabilitation services. 2. The final sentence of the background section refers to the findings of the review informing development of an intervention for stroke survivors - why only stroke survivors? Why not an intervention for acquired neurological conditions? 3. It would be good to have more detail on who conducted the anaysis (i.e. who coded the data and developed the themes and did any other members of the team review this process and outcomes. Some of this detail is mentioned in the author contributions section but it would be good to have some further detail in the data extraction and synthesis section. 4. I am interested to know how involved the advisory group and Stroke Association groups were in the design of the review in terms of patient and public involvement.

REVIEWER	Dr Julie Luker Adjunct Research Fellow, Allied Health & Human Performance, University of South Australia
REVIEW RETURNED	06-Nov-2020

GENERAL COMMENTS	I congratulate the authors on the high quality of this study. Despite a growing recognition of the importance of appropriate information provision to patients with acquired neurological conditions, evidence-based guidance for clinicians has been scarce. The work of these authors is valuable in filling a research evidence gap. The methodology used to synthesize qualitative research findings is exemplary and is clearly described. Their careful work resulted in clear, highly evolved themes that are underpinned by exemplar quotations, and cleverly illustrated in Figure 2. I could find no areas requiring changes and I have no hesitation in recommending this study for publication.

VERSION 1 – AUTHOR RESPONSE

Response to reviewer comments

Reviewer 1	Comments	Author's response
1	My main concern is that there is a lot of reviews on stroke/SCI/brain injury etc with will contain overlapping information e.g., just in stroke literature there is a lot of past reviews. Within the literature there are so many ways the groups could be considered in past	We appreciate the reviewer's comments, however we believe our review does report novel findings. We have examined each of the papers identified by the reviewer and acknowledge that although these reviews, along with others in this area, have considered the broad area of

	evidence it's hard to identify how novel your work is e.g., From a 30 second search on google scholar these reviews looks at HCPS and communication in neuro https://www.tandfonline.com/doi/abs/10.1080/09638288.2020.1822935 https://www.sciencedirect.com/science/article/pii/S0738399120301816 https://www.tandfonline.com/doi/abs/10.1080/09638288.2018.1473508 https://link.springer.com/article/10.1186/s13012-017-0599-3 so that is just one group with out a focus on conditions. Other reviews cover aspects too https://www.tandfonline.com/doi/abs/10.3109/11038128.2010.509887 has a theme on information needs https://onlinelibrary.wiley.com/doi/full/10.1111/j.1369-7625.2011.00683.x has themes related to aspect of care and communication https://journals.plos.org/plosone/article?id=10.1371/journal.pone.0192533 you have other reviews which consider some of the issues you are talking about in other ways e.g., https://www.magonlinelibrary.com/doi/full/10.12968/ijtr.2014.21.5.210 in terms of the focus there is areas you consider within the results which again represent more literature e.g., there is hope (how that is defined varies), expectation (different form hope), recovery (this has its own focus), interactions and communication, goal setting – all of these areas could overlap with your findings.	communication, and others have highlighted more the general areas of communication and information needs in their themes, our review specifically focuses on the area of conversations about recovery/ prognosis and breaking bad news, with a view to making recommendations for clinical practice and developing an intervention. We acknowledge that there are many papers, which briefly touch on this topic, which we believe demonstrates its importance, and the need to consider it in more detail as a topic in its own right.
2	Should a framework like ENTREQ be employed?	An ENTREQ framework has been added in Supplementary File 2, with amendments made to the manuscript to ensure each item has been addressed.
3	Paradigmatic view of the world needs to be added and make sure this links to your quality text?	We do not believe this to be a necessary element; it is not a common inclusion in qualitative systematic reviews.

4	From experience of reviewing qualitative neurological literature most qualitative articles that consider the experience of the condition will include information that meets your eligibility criteria – this means there is likely a lot more than 28 articles that could be eligible - how do you convince me that this is not the case?	We acknowledge that identifying literature in this area is difficult due to a wide variety of search terms used by authors of qualitative publications, and it is of course possible, as with all systematic reviews, that relevant articles could have been missed; we acknowledge this in our limitations. However, we applied broad search criteria, informed by an Information Specialist, conducted backwards and forwards citation searching, and reviewed the full texts of 145 papers. Although a number of these papers appeared initially relevant (i.e. considering communication or patients' experiences), it became apparent during full text review that they did not meet our inclusion criteria in terms of addressing our specific research question (i.e. the views and experiences of patients/ carers/ healthcare professionals in talking specifically about recovery/ prognosis or breaking bad news), and would have contributed little to the synthesis output. Should the editor feel it would benefit the paper, we would be happy to provide the list of full texts and reasons for exclusion as an additional supplementary file.
5	An audit trail to show the synthesis stages to it can be followed?	We have described in detail the stages of synthesis in the methods. The manuscript includes a diagram linking descriptive themes to analytical themes, and references for each point made to allow the primary studies to be identified and thus the developed themes linked back to the data. We have added more detail into the methods section to the involvement of each of the reviewers in this process.
6	The quality assessment – does it impact the synthesis results – if so how can you let the reader know the impact on the themes so what if a theme has weak studies? You rate studies by the ability to fulfil checklist but so what? – does it mean anything for themes?	We have added more detail around our quality assessment and its impact on the synthesis results in the methods section.
7	How does your results account for the various conditions - are you implying that all conditions have the same problems?	In developing our themes, we looked to identify whether they related to one or more of the specific conditions we included in our review. Where this was the case, we identified this in the results section. However, many of the themes cut across

	Could there be a consistent way to identify if the examples you give are from certain conditions, times during the patient journey, delivered by a certain HCP group? Or are you saying all information applies to all groups all the time?	several groups and clinical areas; we have reviewed the results section to check this is the case. In our manuscript, we identify the studies relating to each point made using reference numbers in the main text; the reader may also look to the tables to identify the clinical area or setting from which the information came. For clarity, we have added reference numbers into the tables, so that the reader may more quickly identify the study population reflected in each point made. Should the editor feel it necessary, we could supply an additional supplementary file in which we identify which studies contribute to each theme.
Reviewer 2	Comments	
1	My main query is around what the team are classifying as neurological conditions. Specifically, they report that the papers in the review covered stroke, TBI, SCI, brain tumours and neurological conditions and I would like to know which neurological conditions were included in the review findings. Also, the title and aim of the paper refer to acquired neurological conditions, so it seems a little odd to me that they would then separate some specific neurological conditions in the findings but group others together as neurological conditions. Similarly, in the first theme, the authors discuss how information was often provided during rehabilitation. However, not all people with neurological conditions require or receive rehabilitation (e.g. some forms of epilepsy), so again, some further context regarding the general neurological conditions included in the review would be useful to determine if some patients may be missing out from receiving further information as a result of not necessarily being referred to rehabilitation services.	We thank the reviewer for her comments. We identify in our methods section that the conditions we included are: stroke, traumatic brain injury, spinal cord injury and brain/central nervous system tumours; and identify in the first section of our results the numbers of papers which came from each clinical area. However, we identified some papers, which considered the views of healthcare professionals working across multiple neurological conditions; these were not separated out in the findings of the primary studies, which has meant we are unable to separate our results into separate conditions all of the time. We have clarified this in our methods section. As we describe in point 7 above, we attempted to draw comparisons between conditions where they existed, however common issues were identified across a range of clinical areas.
2	The final sentence of the background section refers to the findings of the review informing development of an intervention for stroke survivors - why only stroke survivors? Why not an	Although our review is likely to be useful for intervention developers working in other neurological conditions, our specific aim is to develop an intervention for use in stroke units as part of an on-going doctoral study;

	intervention for acquired neurological conditions?	we wished to look more broadly at other acquired neurological conditions to see if we could apply learning. We have amended the text in our introduction to indicate that our results are likely to be useful for other intervention developers.
3	It would be good to have more detail on who conducted the anaysis (i.e. who coded the data and developed the themes and did any other members of the team review this process and outcomes. Some of this detail is mentioned in the author contributions section but it would be good to have some further detail in the data extraction and synthesis section.	We have added further detail into the methods section to address this comment.
4	I am interested to know how involved the advisory group and Stroke Association groups were in the design of the review in terms of patient and public involvement.	We have added more detail into the PPI section.
Reviewer 3	No suggested changes	We thank the reviewer for her comments.

VERSION 2 – REVIEW

REVIEWER	Soundy, Andrew University of Birmingham
REVIEW RETURNED	09-Dec-2020

GENERAL COMMENTS	Response to my main concern around past studies was focused towards looking at what articles I identified rather than examining the issue yourself or giving a convincing response was the main concern and brings in to question where there is difference between conditions that will be ignored. Because an exhaustive search was not done I am not sure you can call your review systematic. The response to the paradigm question is disappointing as it would have a significant impact on what you do and how you do it and help the reader understand your decisions and view of the world, as well as how you view quality in reviews. The other responses were given and changes made. I wish you the best for your future research.

REVIEWER	Kılınç, Stephanie Teesside University, Psychology
REVIEW RETURNED	18-Dec-2020

GENERAL COMMENTS	The authors have addressed most of my previous comments. However, I cannot find a satisfactory discussion of the following point that I raised: My main query is around what the team are classifying as neurological conditions. Specifically, they report that the papers in the review covered stroke, TBI, SCI, brain tumours and neurological conditions and I would like to know which neurological conditions were included in the review findings. Also, the title and aim of the paper refer to acquired neurological conditions, so it seems a little odd to me that they would then separate some specific neurological conditions in the findings but group others together as neurological conditions. Similarly, in the first theme, the authors discuss how information was often provided during rehabilitation. However, not all people with neurological conditions require or receive rehabilitation (e.g. some forms of epilepsy), so again, some further context regarding the general neurological conditions included in the review would be useful to determine if some patients may be missing out from receiving further information as a result of not necessarily being referred to rehabilitation services. The choice to categorise the conditions in this way could be commented upon in the background section or even in the strengths/limitations section of the discussion. A note has been included in theme one regarding rehabilitation but it does not address my query.

VERSION 2 – AUTHOR RESPONSE

Response to reviewer comments

Reviewer 1	Comments	Author's response
1	Response to my main concern around past studies was focused towards looking at what articles I identified rather than examining the issue yourself or giving a convincing response was the main concern and brings in to question where there is difference between conditions that will be ignored. Because an exhaustive search was not done I am not sure you can call your review systematic.	We disagree with the reviewer's comment that our review is not systematic: the searches conducted were very comprehensive, and the selection and synthesis were thoroughly systematic, as per the replicable methods detailed.
2	The response to the paradigm question is disappointing as it would have a significant impact on what you do and how you do it and help the reader understand your decisions and view of the world, as well as how you view quality in reviews.	As previously stated, this is an uncommon inclusion in systematic review reporting, and is not a recommended element, e.g. in the ENTREQ checklist. Reporting of paradigmatic stance is only usually reported where it is integral to the approach used to synthesise the results, such as meta-ethnography. As such, we do not believe that this

		inclusion would add anything to the quality of our paper.
Reviewer 2	Comments	
1	My main query is around what the team are classifying as neurological conditions. Specifically, they report that the papers in the review covered stroke, TBI, SCI, brain tumours and neurological conditions and I would like to know which neurological conditions were included in the review findings. Also, the title and aim of the paper refer to acquired neurological conditions, so it seems a little odd to me that they would then separate some specific neurological conditions in the findings but group others together as neurological conditions. Similarly, in the first theme, the authors discuss how information was often provided during rehabilitation. However, not all people with neurological conditions require or receive rehabilitation (e.g. some forms of epilepsy), so again, some further context regarding the general neurological conditions included in the review would be useful to determine if some patients may be missing out from receiving further information as a result of not necessarily being referred to rehabilitation services. The choice to categorise the conditions in this way could be commented upon in the background section or even in the strengths/limitations section of the discussion. A note has been included in theme one regarding rehabilitation but it does not address my query.	We apologise that we failed to fully address this query previously. We included five studies that each reported data from what was generally termed 'multiple neurological conditions'. We were limited by the reporting of the original studies in terms of information about the conditions they included, and how each condition related to the results they presented. However, we have added more detail to the section in the results relating to the population studied in each of the included papers reporting data from multiple neurological conditions. We have also added this to the limitations section of our discussion.